CUSTOMER / PROJECT: Health insurer · Cashless authorization **FDE / TEAM:** Team 6 **DATE / VERSION:** 31 Oct 2026

01 CUSTOMER & STAKEHOLDERS

Who owns the problem? Who uses the process? Who decides?

- F Large health insurance company covering individuals, families and corporate employees.
- F Works with a network of hospitals that offer cashless treatment to eligible policyholders.
- F The COO led the initial discovery meeting and speaks for management.
- F **Process users:** hospitals (submit requests), operations teams (review, clarify, communicate, resolve pending cases) and policyholders (wait for decisions).
- F Stakeholders disagree: some blame technology, others operational processes.
- U Who owns the problem end to end and who makes the final decision? Which operational teams and applications are involved, and who is accountable for each?

02 BUSINESS OBJECTIVE

Why change? What triggered the request? What matters?

- F **Trigger:** increasing complaints over the past several months from policyholders and network hospitals about delays in cashless authorization and claims processing.
- F Management is concerned about operational efficiency, hospital relationships, customer satisfaction and reputation.
- F **COO:** do not invest in another solution without understanding the actual problem; investigate, identify the underlying causes and show where improvement creates the greatest business value.
- U How management ranks its concerns: efficiency, hospital relationships, satisfaction, reputation.
- D Understand the problem and identify what is unknown before recommending any technology or process change.

03 EXPECTED OUTCOMES

What should improve? How will success be measured?

- F The engagement should deliver the underlying causes of delay and where improvements would create the greatest business value.
- F Complaints point to what should improve: time to authorization decision, clarity of request status, and hospitals needing to contact the insurer repeatedly.
- U No targets or success measures are defined. What counts as "longer than expected" is not stated.
- U How success will be measured, and the current level of each measure.
- D Success measures are agreed with the COO once the problem is understood.

04 CURRENT PROCESS / AS-IS

What happens from request to closure? Where are handoffs?

- F Policyholder needs hospitalization → hospital submits a cashless authorization request → insurer reviews the request → insurer communicates its decision to the hospital.
- F Staff also clarify information with hospitals, communicate with them and resolve pending cases. Hospitals sometimes contact the insurer repeatedly for status.
- F Multiple applications and operational teams support the process.
- U The steps from request to closure, handoffs between teams, the application used at each step, and how pending cases are handled. The process is not fully documented.

05 CORE PROBLEM & PAIN POINTS

What is going wrong? What is verified?

- F Complaints about delays in cashless authorization and claims processing are increasing.
- F Some hospitals report authorization requests pending longer than expected.
- F Hospitals sometimes contact the insurer repeatedly to understand a request's status.
- F Policyholders report uncertainty while waiting for authorization decisions.
- F Employees spend considerable time reviewing requests, clarifying information, communicating with hospitals and resolving pending cases.
- U Why delays occur and which parts of the process contribute most: not understood.
- U **Verified causes: none.** Whether technology or operational process is the problem is unproven.

06 BUSINESS IMPACT & BASELINE

How often? What cost, time, risk or rework results?

- F Management fears continued delays could affect operational efficiency, hospital relationships, customer satisfaction and reputation.
- U **Scale and financial impact are unquantified:** how many requests are delayed, by how long, how often, and with what cost, time, risk or rework.
- U How much staff time goes into reviewing, clarifying and resolving pending cases.
- U How many status enquiries hospitals make.
- D No figures are stated until evidence is collected.

07 DATA, SYSTEMS & CONSTRAINTS

What systems, data, rules and dependencies exist?

- F The company has invested in digital platforms to support its cashless insurance operations.
- F Multiple applications support the process.
- U Which applications exist, what data each holds, and what the system capabilities are. These are not fully documented.
- U Rules, dependencies and constraints that apply to authorization decisions: none stated.

08 REQUIREMENTS & MVP OPTIONS

What capabilities might help? What is the smallest, useful pilot?

- F No solution has been approved.
- F Stakeholders disagree on whether the cause is technology or operational process.
- U Which capabilities might help, and what the smallest useful pilot would be. These cannot be defined until the causes are known.
- D No technology or process change is recommended until the underlying causes are established. Both options stay open.

09 VALIDATION & NEXT STEPS

What must be confirmed? Who owns each decision?

- U What "longer than expected" means, and who decides what to do next (the COO led discovery).
- U How often, how long and where in the process requests are delayed.
- U How the current process and systems work, with the operations teams.
- U What network hospitals and policyholders experience while waiting.
- U Whether the technology view or the operational-process view is supported by evidence.
- D The COO is the known decision contact; other decision owners are unknown.